CAMP4 THERAPEUTICS CORPORATION

(NASDAQ: CAMP)

LEERINK PARTNERS

OUTPERFORM
Biopharma / Emerging Rare Disease

Reason for report:
PROPRIETARY INSIGHTS

Tuning In to the FIH Study Design

September 28, 2026

- **Bottom Line: CAMP4's Virtual Analyst Day today at 12:00 p.m. EDT should provide important clarity on the first-in-human (FIH) Phase 1/2 design for CMP-002 in Syngap1-related disorder (SRD),** ahead of the expected 4Q26 clinical start. Notably, CMP-002 is the lead antisense oligonucleotide (ASO) program, and the only regRNA-targeting ASO, in SRD.
- **The FIH study will mark CMP-002's transition from preclinical to clinical development** and provide the first opportunity to generate key POC data that could begin to de-risk the program in SRD, a neurodevelopmental disorder with significant unmet need (see our deep dive HERE). Key areas of focus for us and investors will include dose optimization, biomarker and endpoint selection, and the strategy for assessing efficacy across multiple disease domains, leveraging early lessons from the natural history study (ProMMiS).
- **Additionally, the company will highlight the franchise-building potential of its regRNA platform,** including opportunities to expand into other Syngap-like haploinsufficient developmental and epileptic encephalopathies (DEEs), with CMP-002 providing the first opportunity for clinical validation of the platform. We look forward to further updates at today's Virtual Analyst Day (12:00–1:30 p.m. EDT). We reiterate our Outperform rating; our PT is $14.

Additional details and what to expect:

- **High unmet need:** SRD is a severe, lifelong neurodevelopmental disorder affecting ~21,000 patients across the U.S. and EU5, driven by Syngap1 haploinsufficiency and characterized by significant neurological and functional impairment, high treatment burden, and no approved disease-modifying therapies.
- **Study clearance:** CAMP4 received clearance to conduct its Phase 1/2 trial in Australia, Argentina and the UK to support initiation of the study in 4Q26 across 10-12 sites.
    - Regulatory filings in the EU remain under review based on the latest disclosure.

Key Stats

S&P 600 Health Care Index:	**3,811.11**
Price:	**$3.92**
Price Target:	$14.00
52-Week High – Low:	$7.75 - $2.63
Shares Outstanding (mil):	90.7
Market Capitalization (mil):	$355.5
Cash Per Share:	$1.43
Dividend (ann):	-

	Dec Yr		
	2025A	**2026E**	**2027E**
Q1	$0.9	$1.3A	--
Q2	$1.5	$1.8A	--
Q3	$0.8	$1.8	--
Q4	$0.3	$1.8	--
FY Rev	$3.5	$6.7	$8.0
Q1	($0.62)	($0.32)A	--
Q2	($0.62)	($0.58)A	--
Q3	($0.55)	($0.21)	--
Q4	($0.85)	($0.21)	--
FY EPS	($2.65)	($1.32)	($0.90)
P/E	NM	NM	NM

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Source: Company Information and Leerink Partners LLC Research.
Revenue in $ millions. EPS - Diluted ($).

Please refer to page 12 for Important Disclosures, Price Charts and Analyst Certification.

- **Patient population:** The study will initially enroll a sentinel patient aged 8–10 years before expanding to patients as young as 2–3 years, where earlier intervention could improve signal detection by enabling treatment before substantial neurological impairment occurs.
- **Study design:** Placebo-controlled MAD design with 3 cohorts. Management indicated that the study design is adaptive, which we believe would help shift enrollment towards the dose with the best efficacy/safety profile in pediatric patients. We look forward to additional details later today.
    - Given CMP-002's PK/PD profile is broadly consistent with other oligonucleotides, management believes the NHP data provide a reasonable basis for anticipating translation to humans.
    - Management believes the placebo-controlled design could facilitate a registrational path and potentially support accelerated/conditional approval.
- **Endpoints:** The company is including efficacy assessments in the Phase 1/2 study across all major disease domains, including seizures, sleep, motor, communication, behavior and development.
    - Early data from ProMMiS validated a protocol with a full battery of assessments that is feasible in SRD patients.
    - **Seizures:** Countable seizures appear to be present in approximately 20%–45% of SRD patients aged 2–10 years (according to ProMMiS); therefore, we will be watching for further details on CAMP's planned approach to seizure assessment.
    - **Cognition and neurodevelopmental endpoints:** The company is planning to measure Bayley, Vineland and GMFM-66 with early ProMMis data demonstrating no significant floor or ceiling effects with the raw scores to date.
    - **Sleep:** The company is planning to use EEG and video EEG.
    - **Patient-reported outcomes** and diaries by incorporating input from patient organizations.
    - **Timelines:** Importantly, assessment timelines are expected to vary across the different outcomes, and today's update should provide greater clarity on this important aspect of the study design.
- **Cash position:** As of end of June, CAMP had ~$130M in proforma cash extending runway through the end of 2028.
- *See within for additional details.*

INVESTMENT THESIS

- SRD represents a high-unmet-need genetic disorder driven by SYNGAP1 haploinsufficiency, with no approved disease-modifying therapies and current treatment limited to symptomatic management.
- CMP-002 offers a compelling disease-modifying approach by targeting regRNA to increase endogenous SYNGAP1 expression and restore protein levels; importantly, the program is currently leading the competitive development landscape.
- CMP-002 leverages an established CNS therapeutic modality, with intrathecal ASO delivery clinically validated by approved therapies including Spinraza and Qalsody, helping reduce modality and delivery risk.
- Beyond CMP-002, we see meaningful unmodeled upside from potential BD/licensing of CMP-001 in UCD and additional programs generated through CAMP's proprietary RAP platform, which is very versatile. We currently ascribe no explicit value to these opportunities despite clinical data already generated for CMP-001.

VALUATION

Our $14 PT is based on a SOTP analysis that forecasts cash flows into the early 2040s, with a 12% discount rate.

RISKS TO VALUATION

- Disease heterogeneity could make it challenging to demonstrate a clear and consistent efficacy signal across patients.
- Clinical benefit could be largely confined to seizure reduction, with limited improvement in more meaningful behavioral and cognitive outcomes, potentially constraining adoption.
- Unmodeled competitive entrants, including potentially disruptive therapeutic modalities, could reduce CMP-002’s addressable market and commercial opportunity.
- Unexpected safety findings, particularly those that limit dosing or chronic administration, could negatively impact the program’s development and commercial potential.

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CMP-002 Phase 1/2 trial design: What We Know and What to Expect

CMP-002 is advancing toward a global Phase 1/2 trial with initiation expected in 4Q26

Phase 1/2 study designed to establish early PoC across key SYNGAP1 disease domains

- United Kingdom's MRHA and Australia's TGA/ HREC clearance in addition to clearance in Argentina support initiation of the global Phase 1/2 study
- Additional regulatory submissions in EU are underway.

Phase 1/2 endpoint categories



Potential study design

- **Global study designed for rapid enrollment, moving straight to MAD,** with multiple dose cohorts and an aim to start in pediatric patients
- The goal is to establish safety and identify an optimal biological dose in pediatric patients and identify an optimal biological dose
- **Efficacy assessments across all major disease domains,** including seizures, sleep, motor, communication, behavior and development/ cognition
- **Enriched genotype / phenotype pediatric population** focused on a genotype / phenotype-enriched population representative of the majority of patients with classic SYNGAP1 haploinsufficiency and with features including seizures, impaired sleep and limited speech
- **Endpoints mapped to natural history for additional control**, with ProMMiS (>100 patients) and CURE SYNGAP1 work providing potentially valuable context
- Data from the trial should help **inform optimal biological dose selection**
- **The company is conducting an OLE study to demonstrate long-term disease-modifying benefit;** ultimately, proof of concept (PoC) will depend on a credible efficacy signal against a heterogeneous natural-history backdrop.

We look forward to the initiation of the global FIH Phase 1/2 trial, which should provide key PoC data to partially de-risk the asset in this disease with high unmet need.

FIH, First-in-human, OLE, open-label extension, ProMMis, prospective multidisciplinary multisite study for clinical excellence; MHRA, Medicines and Healthcare Products Regulatory Agency

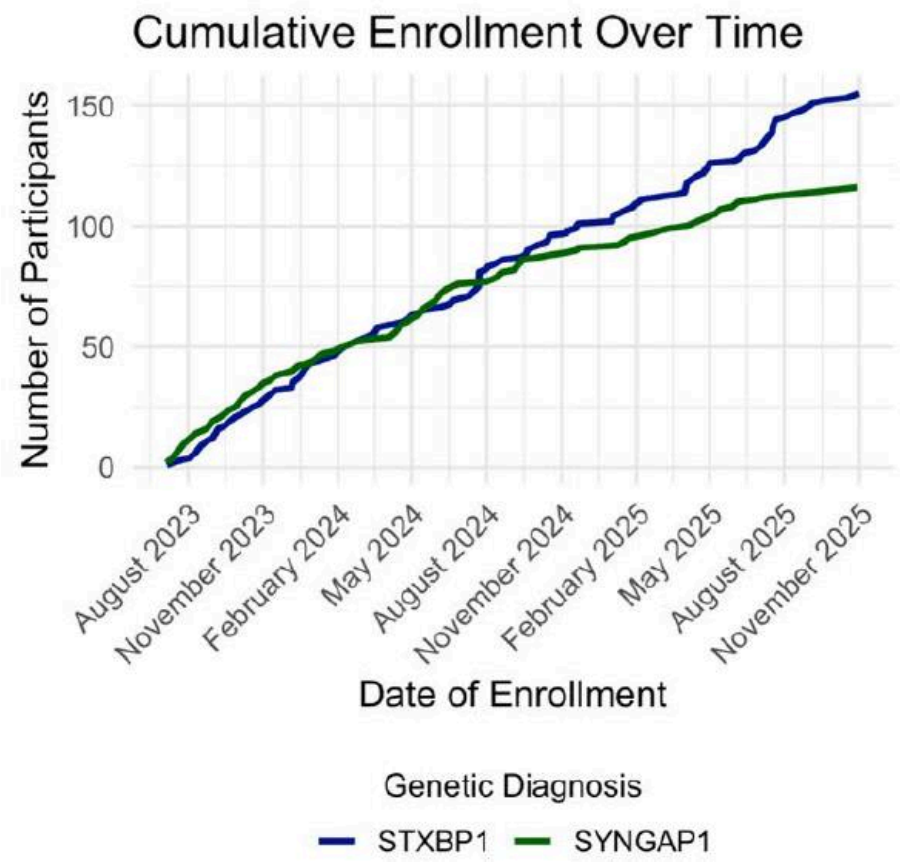
Sources: Company reports and publications; Leerink Partners Research

ProMMiS has assembled a broad SYNGAP1 population to inform early trial design

The cohort is sufficiently broad to guide patient selection and endpoint feasibility, although longitudinal follow-up remains limited

- **159 individuals** with SYNGAP1-RD enrolled in the prospective ProMMiS natural-history study; the broader cohort spans infancy through adulthood
- **Longitudinal maturity is emerging:** 73 participants had a second assessment, and 32 had more than two evaluations, supporting preliminary trajectory analyses but not yet a definitive untreated disease slope
- **Seizure-reconstruction dataset:** 100 individuals contributed approximately 950 patient-years of monthly seizure-history data; 84% had experienced seizures, with median onset at 2.2 years
- **Functional-assessment dataset:** 115 individuals completed the GMFM-66, with 99% able to complete the measure and minimal floor or ceiling effects.

Dataset	Available population	Relevance for CAMP
Total SYNGAP1 enrollment	159	Recruitment feasibility and population breadth
Repeat assessment	73	Preliminary longitudinal change
>2 assessments	32	Early trajectory estimation
Seizure reconstruction	100	Seizure timing and burden
GMFM-66	115	Functional endpoint feasibility



Cumulative enrollment in the ProMMiS natural-history study from 2023–2025, showing steady recruitment of individuals with SYNGAP1-RD and growth of the dataset available to support clinical trial planning

ProMMiS is already useful for defining the Phase 1/2 target population, baseline heterogeneity, and feasible outcome measures; additional longitudinal follow-up is needed to quantify expected placebo-group change.

GMFM-66, Gross Motor Function Measure-66; NHS, natural history study; ProMMiS, Prospective Multicenter Study of SYNGAP1; RD, related disorder
Sources: McKee, Epilepsia, 2026; ProMMiS preliminary natural-history data; Company reports and publications; Leerink Partners Research

Candidate developmental endpoints for SYNGAP1 clinical trials

No single instrument captures the full SYNGAP1 phenotype; endpoint selection should balance developmental breadth, real-world function, motor sensitivity, age range, and feasibility

- The measures assess different levels of function, from developmental skills observed in clinic to adaptive abilities demonstrated in daily life
- Bayley-4, Vineland-3, and GMFM-66 assess complementary aspects of SYNGAP1-related impairment; endpoint selection should reflect age, developmental severity, and the expected treatment effect.

Endpoint	What it measures	Scale	Strengths	Limitations
Bayley-4	Cognition, language, motor, social-emotional, and adaptive behavior; ages 16 days-42 months	Composite: ~40–160; 100 = age-expected performance; higher = better Subtests: 1–19; 1 = lowest, 19 = highest	**Clinician outcome measure;** useful in younger children	Limited age range; floor/ ceiling effects; administration burden
Vineland-3	Everyday adaptive behavior: communication, daily living skills, socialization; optional motor domain; birth–90 years	Domain/ ABC scores: 20–160; 100 = age-expected performance; higher = better. Item scores: 0 = never/ rarely performs, 2 = usually performs	**Caregiver-reported outcome;** broad age range; captures meaningful real-world function and caregiver-relevant change	Informant bias; influenced by environment and therapy; may be less sensitive to short-term change
GMFM-66	Gross motor function across lying/rolling, sitting, crawling/kneeling, standing, and walking/running/jumping	Item: 0 = does not initiate; 3 = completes task. Total: 0–100; 0 = lowest ability, 100 = highest ability	**Clinician outcome measure;** objective, motor-specific, Rasch-scaled measure designed to detect longitudinal change	Does not assess cognition, language, behavior, or seizures; developed primarily in cerebral palsy

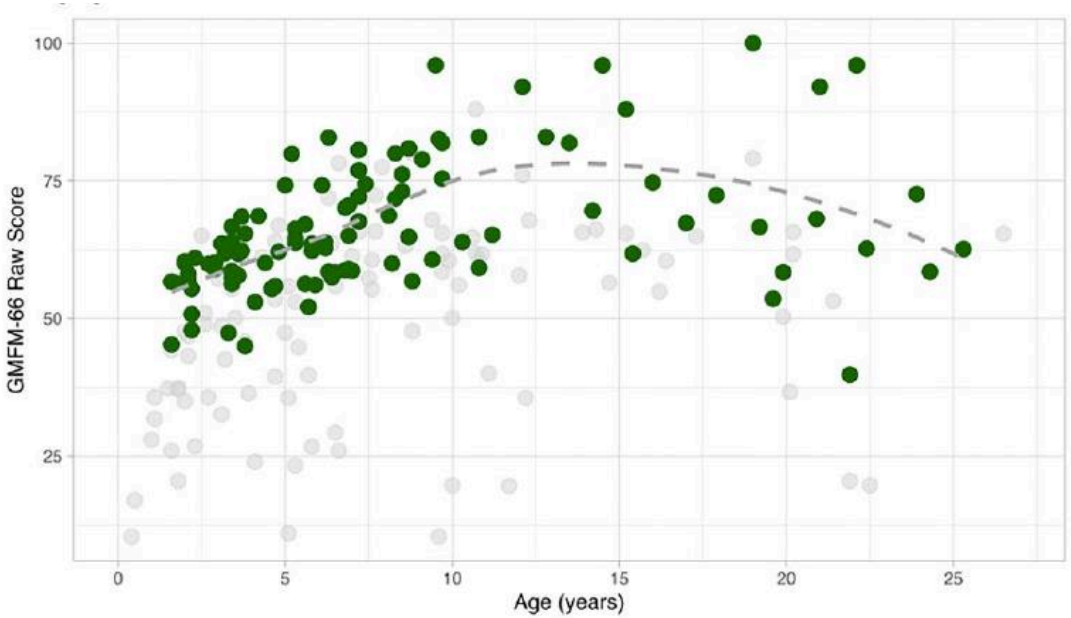
A balanced SYNGAP1 endpoint strategy could use Bayley-4 in younger children, Vineland-3 as the broad real-world functional measure, and GMFM-66 as a complementary objective motor endpoint rather than relying on any single scale.

Sources: Bayley, Pearson Assessment Manual, 2019; Sparrow, Pearson Assessment Manual, 2016; Russell, Physical Therapy, 2000; Company reports and publications; Leerink Partners Research

GMFM-66 captures a broad functional range across the SYNGAP1 population

The GMFM-66 is feasible across ages and disease severity, making it a good candidate for functional assessment in CAMP4's Phase 1/2 studies

- **High feasibility:** 99% of SYNGAP1 participants assessed at baseline completed the GMFM-66 (115/ 116)
- **Broad dynamic range:** scores span approximately 40-100 across the cohort, allowing differentiation of functional ability without a major floor or ceiling effect
- **Age-related pattern is visible:** scores generally increase during early childhood before flattening in adolescence; however, the cross-sectional dataset cannot establish individual patient trajectories
- **Key next step is responsiveness:** longitudinal GMFM-66 data, supported by Vineland-3 and caregiver-reported outcomes, are needed to determine whether changes are clinically meaningful to families.



Initial GMFM-66 scores by age in individuals with SYNGAP1-related disorder (green) and STXBP1-related disorder (gray). The SYNGAP1 population demonstrates a broad range of gross-motor function with minimal floor or ceiling effects; the dashed line represents the fitted age-related trend.

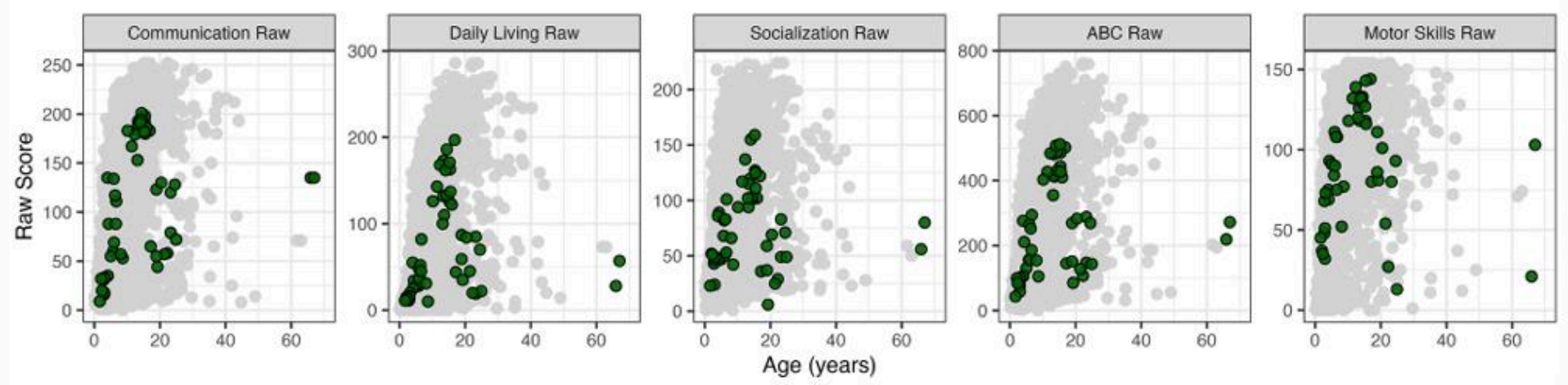
GMFM-66 is currently one the strongest candidate functional endpoint for early CAMP4 trials, but longitudinal data and caregiver anchors are required to define meaningful change and support powering decisions.

GMFM-66, Gross Motor Function Measure-66; qEEG, quantitative electroencephalography; Vineland-3, Vineland Adaptive Behavior Scales, Third Edition; PROM, patient-/parent-reported outcome measure.

Sources: McKee, Epilepsia, 2026; ProMMiS preliminary natural-history data; Company reports and publications; Leerink Partners Research

Vineland-3 captures parent impressions of adaptive behavior across domains

- **Feasibility:** emerging data from ProMMis were merged with existing Vineland-3 data from the Simons Searchlight database for individuals with SRD (n=39) using the RARE-X platform, and preliminary analysis suggests that this measure is appropriate and feasible
- **Dynamic range:** the data is insufficient to assess floor effects
- **Vineland-3 includes 5 domains:** communication, daily living, socialization, ABC composite (sum of the prior 3) and motor skills.



Vineland-3 scores by age from Simons Searchlight in individuals with SYNGAP1-related disorder (green) vs all others with rare neurodevelopmental disease in the Simons registry (gray).

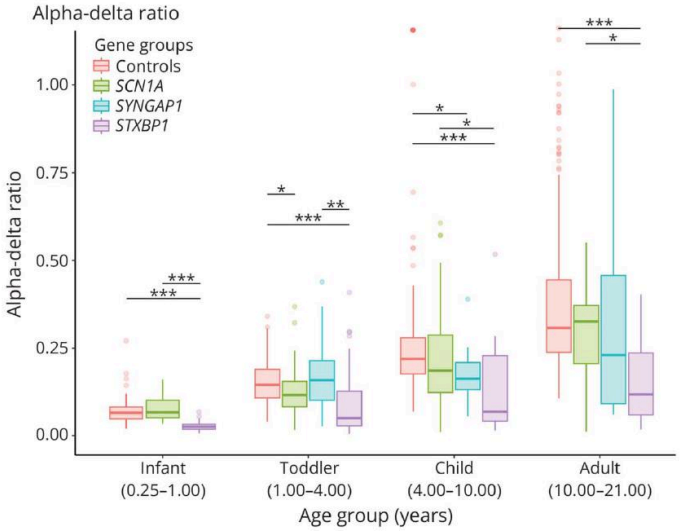
Vineland-3 is another strong candidate functional endpoint for early CAMP4 trials, with the endpoint being relevant to assess the ability of CMP-002 to improve or preserve real-world functioning across multiple domains, not just a single symptom.

Vineland-3, Vineland Adaptive Behavior Scales, Third Edition
Sources: McKee, Epilepsia, 2026; ProMMiS preliminary natural-history data; Company reports and publications; Leerink Partners Research

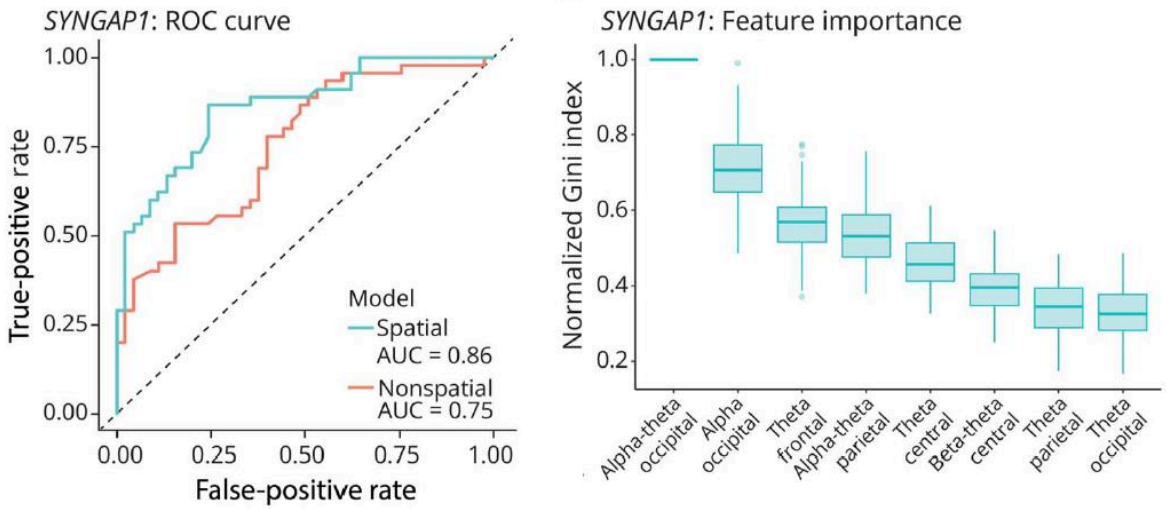
Reduced fast-to-slow EEG activity in SYNGAP1 may provide an objective biomarker

SYNGAP1 shows evidence of abnormal EEG spectral dynamics, with a reduced alpha–delta ratio in children and alpha–theta features among the strongest variables distinguishing SYNGAP1 from controls

- In 46 EEGs from 21 individuals with SYNGAP1, the alpha–delta ratio was lower than controls during childhood (p = 0.043; Cohen's d = −0.62), consistent with increased relative slow-wave activity
- The alpha–theta ratio was among the top features in machine-learning models distinguishing SYNGAP1 from controls; the strongest SYNGAP1 features localized primarily to occipital electrodes
- qEEG could complement seizure, developmental, and motor endpoints as an objective biomarker, but these findings are currently exploratory and retrospective; treatment responsiveness has not yet been established.



Alpha–delta ratio was lower in children with SYNGAP1 than in age-comparable controls, supporting increased EEG slowing in SYNGAP1-related epilepsy.



Spatial spectral features distinguished SYNGAP1 from controls with an AUC of 0.86; alpha–theta features, particularly from occipital electrodes, were among the most informative predictors.

qEEG identifies abnormal fast-to-slow spectral dynamics in SYNGAP1, with reduced alpha–delta ratios and alpha–theta features contributing to disease classification; prospective studies are needed to determine whether these measures respond to treatment.

AUC, area under the curve; EEG, electroencephalography; GMFM-66, 66-item Gross Motor Function Measure; qEEG, quantitative electroencephalography; SYNGAP1, synaptic Ras GTPase-activating protein 1

Sources: Galer, Neurology, 2025; Company reports and publications; Leerink Partners Research

CAMP4 Therapeutics (CAMP) Income Statement

Camp4 - Income Statement

($ Millions, except per-share data)

CAMP P&L	2024	1Q25	2Q25	3Q25	4Q25	2025	1Q26A	2Q26A	3Q26E	4Q26E	2026E	2027E	2028E	2029E	2030E	2031E
CMP - SYNGAP	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0
Research & Collaboration revenue	0.7	0.9	1.5	0.8	0.3	3.5	1.3	1.8	1.8	1.8	6.7	8.0	2.8	0.0	0.0	0.0
Total revenue	**0.7**	**0.9**	**1.5**	**0.8**	**0.3**	**3.5**	**1.3**	**1.8**	**1.8**	**1.8**	**6.7**	**8.0**	**2.8**	**0.0**	**0.0**	**0.0**
Consensus																
COCGS	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0
Gross Profit	0.7	0.9	1.5	0.8	0.3	3.5	1.3	1.8	1.8	1.8	6.7	8.0	2.8	0.0	0.0	0.0
R&D	38.8	10.1	10.3	9.4	8.4	38.2	10.2	10.8	10.9	11.2	43.0	50.0	50.0	65.0	85.0	85.0
SG&A	14.9	3.8	4.2	4.6	4.8	17.4	4.2	4.3	4.7	4.8	18.0	19.8	22.1	23.2	24.4	37.3
Impairment of right-of-use asset		0.0	0.0	0.5	0.0	0.5										
Total operating expenses	**53.7**	**14.0**	**14.5**	**14.4**	**13.1**	**56.1**	**14.4**	**15.2**	**15.5**	**15.9**	**61.0**	**69.7**	**72.1**	**88.2**	**109.4**	**122.3**
Operating income	**-53.1**	**-13.1**	**-13.0**	**-13.7**	**-12.8**	**-52.6**	**-13.1**	**-13.4**	**-13.7**	**-14.1**	**-54.3**	**-61.7**	**-69.3**	**-88.2**	**-109.4**	**-122.3**
Total non operating income	**1.3**	**0.7**	**0.4**	**-1.4**	**-27.5**	**-27.8**	**-5.3**	**-20.2**	**0.7**	**1.1**	**-23.6**	**4.5**	**5.0**	**7.5**	**7.0**	**3.1**
(Pre-tax loss) / Pre-tax income	**-51.8**	**-12.4**	**-12.6**	**-15.1**	**-40.3**	**-80.4**	**-18.3**	**-33.5**	**-13.0**	**-13.0**	**-77.9**	**-57.2**	**-64.3**	**-80.7**	**-102.4**	**-119.2**
Taxes	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0
Tax Rate																
Net Income (loss)	**-51.8**	**-12.4**	**-12.6**	**-15.1**	**-40.3**	**-80.4**	**-18.3**	**-33.5**	**-13.0**	**-13.0**	**-77.9**	**-57.2**	**-64.3**	**-80.7**	**-102.4**	**-119.2**
Avg Shares - Basic	4.7	20.2	20.2	27.3	47.5	30.4	57.9	57.9	60.9	62.9	59.9	63.3	107.4	107.8	108.2	108.6
Avg Shares - Fully Diluted	6.8	22.4	23.2	30.2	50.4	34.8	62.3	64.8	80.7	90.9	74.7	91.3	135.4	135.8	136.2	136.6
EPS (basic)	**-$11.04**	**-$0.62**	**-$0.62**	**-$0.55**	**-$0.85**	**-$2.65**	**-$0.32**	**-$0.58**	**-$0.21**	**-$0.21**	**-$1.32**	**-$0.90**	**-$0.60**	**-$0.75**	**-$0.95**	**-$1.10**

Source: SEC Filings, company reports and Leerink Partners Research Estimates

Source: Company reports, Leerink Partners estimates

Margin Analysis	2024	1Q25	2Q25	3Q25	4Q25	2025	1Q26A	2Q26A	3Q26E	4Q26E	2026E	2027E	2028E	2029E	2030E	2031E
Revenues	100.0%	100.0%	100.0%	100.0%	100.0%	100.0%	100.0%	100.0%	100.0%	100.0%	100.0%	100.0%	100.0%	100.0%	100.0%	100.0%
COGS																
Gross Margin																
SG&A																
SG&A (Excl CD20 Revs)																
R&D																
Operating Margin																
Pretax Margin																
Net Margin																

% Change	2024	1Q25	2Q25	3Q25	4Q25	2025	1Q26A	2Q26A	3Q26E	4Q26E	2026E	2027E	2028E	2029E	2030E	2031E
Revenues																
COGS																
Gross Profit																
SG&A						16.3%	10.3%	3.8%	1.5%	-0.1%	3.5%	370.3%	409.7%	398.4%	412.3%	107.4%
R&D						-1.6%	0.1%	4.6%	16.2%	33.6%	12.6%	391.8%	361.8%	497.8%	661.0%	97.6%
Total Expenses						4.3%	2.9%	4.4%	7.5%	21.4%	8.8%	385.5%	375.5%	468.0%	586.6%	100.5%
Operating Income						-1.0%	-0.2%	2.7%	0.6%	10.6%	3.3%	372.3%	417.8%	542.4%	674.1%	125.1%
Pretax Income						55.2%	47.4%	166.4%	-13.7%	-67.6%	-3.1%	212.3%	91.7%	519.4%	684.7%	52.9%
Net Income						55.2%	47.4%	166.4%	-13.7%	-67.6%	-3.1%	212.3%	91.7%	519.4%	684.7%	52.9%
EPS						-76.0%	-48.7%	-7.3%	-61.3%	-75.6%	-50.2%	186.0%	3.4%	249.7%	355.8%	-16.7%

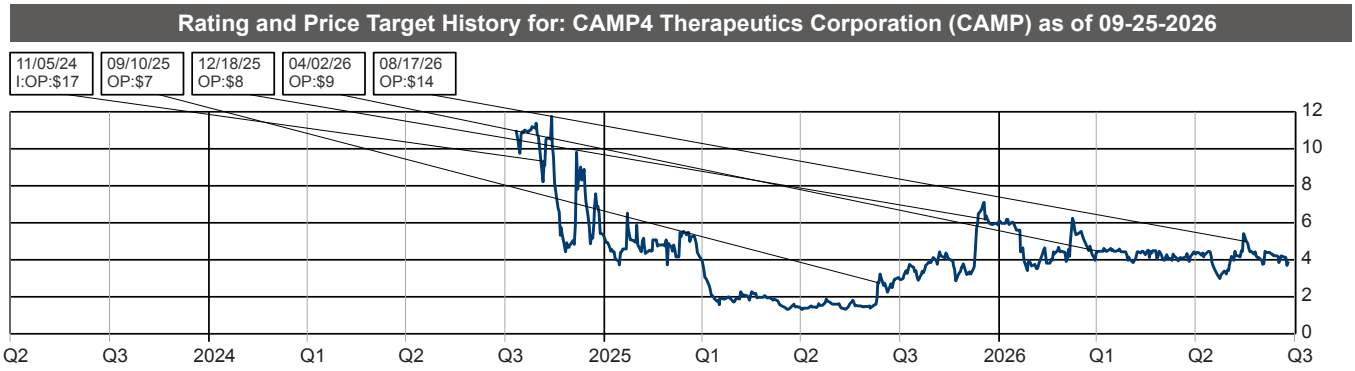
Sources: Leerink Partners Research

Disclosures Appendix

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Revised Distribution: September 28, 2026 6:33 A.M. EDT.

Analyst Certification

I, Basma Radwan, Ph.D., certify that the views expressed in this report accurately reflect my views and that no part of my compensation was, is, or will be directly related to the specific recommendation or views contained in this report.



OP = Outperform MP = Market Perform UP = Underperform D = Drop Coverage I = Initiate SC = Suspended Coverage

Created by: BlueMatrix

Valuation

Our $14 PT is based on a SOTP analysis that forecasts cash flows into the early 2040s, with a 12% discount rate.

Risks to Valuation

- Disease heterogeneity could make it challenging to demonstrate a clear and consistent efficacy signal across patients.
- Clinical benefit could be largely confined to seizure reduction, with limited improvement in more meaningful behavioral and cognitive outcomes, potentially constraining adoption.
- Unmodeled competitive entrants, including potentially disruptive therapeutic modalities, could reduce CMP-002's addressable market and commercial opportunity.
- Unexpected safety findings, particularly those that limit dosing or chronic administration, could negatively impact the program's development and commercial potential.

Distribution of Ratings/Investment Banking Services (IB) as of 06/30/26				
			IB Serv./Past 12 Mos.	
Rating	Count	Percent	Count	Percent
BUY [OP]	241	80.3	142	58.9
HOLD [MP]	57	19.0	14	24.6
SELL [UP]	2	0.7	0	0

Explanation of Ratings

Outperform (Buy): We expect this stock to outperform its benchmark over the next 12 months.

Market Perform (Hold/Neutral): We expect this stock to perform in line with its benchmark over the next 12 months.

Underperform (Sell): We expect this stock to underperform its benchmark over the next 12 months.

The degree of outperformance or underperformance required to warrant an Outperform or an Underperform rating should be commensurate with the risk profile of the company.

For the purposes of these definitions the relevant benchmark for "Leerink Partners" branded healthcare and life sciences equity research will be the S&P 600® Health Care Index for issuers with a market capitalization of less than $2 billion and the S&P 500® Health Care Index for issuers with a market capitalization over $2 billion.

Important Disclosures

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